Email from Handling Editor Prof. Alexander Tsai

Dear Ms. Edlund,

Thank you for submitting your manuscript to SSM - Mental Health.

I have completed my evaluation of your manuscript. The reviewers recommend reconsideration of your manuscript following revision. I invite you to resubmit your manuscript.

To view your reviewer feedback, please log in as an author at https://www.editorialmanager.com/ssmmh/ and navigate to your manuscript in the " Submissions Needing Revision " folder under the Author Main Menu.

When revising your manuscript, please consider all issues mentioned in the reviewers' comments carefully. Please note that your revised submission may need to be re-reviewed.

Please resubmit your revised manuscript by Nov 14, 2026.

In assembling your revision, please prepare the following documents:

1. A rebuttal memo giving a point-by-point response to the reviewers' and Editors' concerns. In order to expedite the processing of the revised manuscript, please be as specific as possible in your response; clearly differentiate the reviewer/Editor comments from your replies, either with use of fonts (e.g., reviewer comments in boldface and author responses in non-boldface) or tables (e.g., reviewer comments in column 1 and author responses in column 2); and note the page number in the revised manuscript where the revised text can be found. Should you disagree with the criticisms raised, please provide explanations. Please do not invoke the word count as an explanation for not providing an elaboration in response to a reviewer comment; in general, unless your revisions substantially increase the length of the manuscript, such additions would be accommodated.

2. A revised manuscript, clearly identifying all the changes made in the document (e.g., using the "track changes" function or highlighted text). Please address all of the concerns raised by the reviewers within the manuscript.

3. A "clean" version of your revised manuscript where the changes are not marked.

If we have not heard back from you by Nov 14, 2026, we will assume you do not wish to submit a revision and your manuscript will be withdrawn. If you feel you will need more time to revise and re-submit your paper, please let us know.

Editor and Reviewer Comments:

Editor:

I was on the fence between reject vs. revise. The problems are potentially fixable.

Major comments:
1. I think you need to distinguish much more clearly between the effect of TANF receipt, the effect of benefit generosity among TANF-eligible families, and the association between a state's TANF policy environment with mental health among all children living below 200% FPL. The current exposure is "a gross state-level measure" that is a standardized maximum benefit for a hypothetical family of three with no income, but the analytic sample includes many families who will not receive TANF as well as others whose eligibility and benefit amounts differ substantially.

I don't think this necessarily creates a simple measurement error that will bias your estimates toward zero. But I do think it changes your estimand. You could reframe the primary analysis as estimating associations between state TANF generosity with child mental health in low-income populations, not the effect of TANF receipt or the effect of a family's actual benefit amount. If NSCH variables permit a more family-specific policy measure based on household size and income category, that would be a useful sensitivity analysis, but I don't think you necessarily need to do that.

2. The discussion about identification needs to be beefed up. State and year fixed effects remove time-invariant state characteristics and common national shocks, but they don't purge your estimates of state-specific time-varying confounding. e.g. states changing TANF benefits could also have changed benefit access, work requirements, sanctions, other antipoverty policies, or behavioral health policies, or they could have experienced changing economic conditions. (I couldn't say offhand either way.) So I think your statement that TWFE models "likely present more robust estimates of how changing a state's TANF cash benefit generosity impacts children's mental health" is probably stronger than the data will bear.

I agree with R2 that you should more fully discuss continuous-treatment DD methods and assumptions. Callaway, Goodman-Bacon, and Sant'Anna (NBER 2024/AER 2026) show that linear TWFE coefficients for continuous exposures can be difficult to interpret without stronger assumptions about exposure-effect heterogeneity and selection across exposure ranges. You might also find Roth, Sant'Anna, Bilinski, and Poe (J Econometrics 2023) helpful for a survey of the assumptions, diagnostics, and limitations of DD analyses.

3. Is there any chance that your findings could be driven by a single state or a few states that had anomalously large increases in TANF benefits?

In the appendix can you please report the distribution of within state changes? Then readers can see the number of states with no change vs. modest change vs. substantial change. A state

whose maximum TANF benefit did not change from 2015 through 2021 does not contribute much, if any, identifying variation to the TANF coefficient in a state-fixed-effects model.

To address this possibility you could conduct a series of leave-one-state-out sensitivity analyses, ie. re-estimate the model 51 times, each time omitting all observations from one state (or DC), then present the distribution of the resulting TANF coefficients.

4. In terms of temporal ordering, I see that the one-year policy lag precedes the survey, but caregiver-reported diagnoses, especially ADHD, may have originated well before the lag period. I think you should also discuss more directly the fact that diagnosis-based outcomes reflect both underlying morbidity and differential access to assessment and care.

5. You describe the age/sex-stratified TWFE estimates as "protective" at multiple points throughout. But in the appendix there appears to be statistically and substantively significant adverse effects for some subgroups. (I wouldn't necessarily overinterpret those--could just be due to chance) But I would not necessarily describe the within-state results simply as "null" or as supporting a generally protective effect.

Minor comments:
6. Do you want to consider a wild cluster bootstrap sensitivity analysis? (Cameron Gelbach Miller REStat 2008). 50 states is probably enough to stick with your cluster robust SEs, but the effective number of clusters contributing identifying variation is substantially lower (benefits increased in only 27 states).

7. I think R1's suggestion to use the terminology "children living in low-income households" is reasonable.

Reviewer #1:
The article presents associations between TANF generosity and children's mental health. I found the article to be well-written and interesting. The analyses were rigorous, and I applaud the authors for their work. Given this, I have mostly minor semantic comments for the authors to consider.

1. I found the language "experiencing" low income to be a bit strange as a reader. I recognize this might have been driven by a desire to use person-first language, but to me it felt a bit overly academic. I would consider just "low income families" or "living in low income households."

2. One limitation of using data from the NSCH is that the authors were not able to verify that the included children received TANF benefits. The authors discuss the limited reach of TANF, but I would encourage them to explicitly name what this might mean for their study: the children included in their analysis may or may not have been receiving TANF benefits.
3. When recapping the results in the first paragraph of the discussion, I would encourage the authors to also restate which models they are referencing (e.g., "In cross sectional models of a nationally representative sample of children experiencing low income, a $100 increase in

maximum monthly TANF cash benefits reduced the probabilities of externalizing disorders such as ADHD/ADD and, to a marginal degree, behavioral problems…")

4. I would suggest the authors expand on this point in the Discussion: "Early childhood also represents a sensitive developmental period marked by heightened reliance on caregivers and the home environment. Our finding that TANF generosity is protective against anxiety only in earlier childhood is thus in line with FSM hypotheses (Fields et al., 2021)." Can you add more about how this is in line with the FSM? Does the FSM differentiate between developmental periods?

5. I also think it would be appropriate to expand discussion of the limitations of the outcome variable. First, as the authors note in the introduction, mental health care is difficult to access nationally and many youth may not receive diagnoses. As such, relying on professional diagnoses may miss many youth, especially considering that lower-income youth may be less likely to access care.

6. Relatedly, ADHD is influenced by both the environment and biology. I would argue it is more heavily biological than the other disorders examined). ADHD has also been particularly medicalized, such that individuals diagnosed with ADHD often identify with that diagnosis for life (as opposed to symptoms of depression, anxiety, or other externalizing behavioral problems that may remit over time). What I think that means for the survey is that even if youth's symptoms did improve with additional cash assistance, that is unlikely to be reflected in the current outcome measure. Though significant cross-sectional associations were found for ADHD, I think a little more nuanced discussion of the particularities of this disorder as opposed to the others may be warranted.

Reviewer #2:
This paper attempts to determine the effect of TANF (measured continuously in dollars) on mental health outcomes in children using the NSCH (2016-2022). It assigns "maximum monthly cash benefit provided to a family of three, with no income, residing in each state's most populous area" (with some adjustments where "the reported maximum monthly benefit varied by eligibility group, we included the values affecting the majority of the TANF caseload in each state"). This measure is a gross state-level measure. Thus, it will not vary by the actual composition of each family within each state, creating measurement error that will bias the estimates towards zero.

In addition, there is a growing literature on the use of continuous treatment in difference-in-differences contexts, but none of this literature is discussed or cited in the methods section. There are also no tests of model validity. The reader has no clarity what estimator was used or what software was used. There is a large literature about how TWFE can give biased results if not performed correctly and this needs to be explicitly addressed. The current methods presentation does not allow the reader to determine the validity of the results.